

MedVoice AI

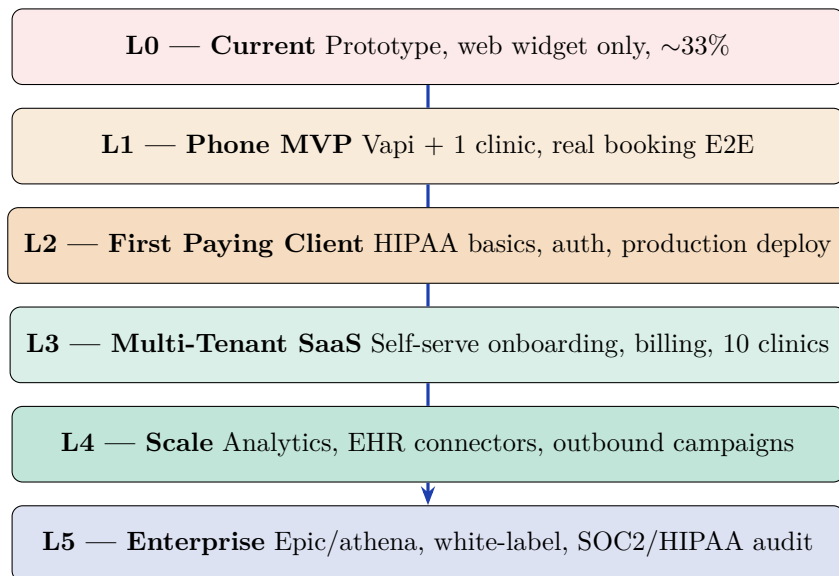
Complete Project Roadmap

Product Roadmap — Level 0 to Production

June 2026

1 Roadmap Overview

Five levels from current prototype to scalable SaaS. Each level has hard exit criteria before advancing.



2 Level 0 — Current State (Complete)

Duration: Done (Passes 1–8)

Delivered:

- Web voice/chat widget with full conversation engine
- Multi-tenant config via `?client=`
- Ollama LLM proxy, emergency detection (verified)
- Lead capture + HMAC webhooks (browser outbox)
- SQLite persistence + React dashboard (wired)
- Calendar API code (disabled in config)

Blockers to L1: No live phone; no Vapi; calendar not E2E verified.

3 Level 1 — Phone MVP (Weeks 1–2)

Goal: One real clinic answers phone calls via Vapi end-to-end.

Task	Owner	Est.
Vapi account + provider keys	Dev	0.5d
/api/vapi/* routes (3 handlers)	Dev	2d
Tool definitions (calendar, lead, emergency)	Dev	1d
Enable Google Calendar + E2E test	Dev	1d
Twilio number → Vapi routing	Dev	0.5d
Server-side webhook worker	Dev	1d
Pilot clinic config + prompt tuning	Dev + Clinic	2d
Total		~8 days

Exit criteria:

1. Inbound call books real appointment on Google Calendar
2. Transcript appears in dashboard within 30s of hang-up
3. Emergency keyword triggers log + transfer offer
4. Zero browser-tab dependency for phone calls

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4 Level 2 — First Paying Client (Weeks 3–6)

Goal: HIPAA-minimum viable product; one clinic pays monthly.

Task	Priority	Est.
HIPAA-eligible hosting (AWS/Railway BAA)	P0	3d
Sign BAAs (Vapi, Twilio, Deepgram, OpenAI, ElevenLabs)	P0	5d
Encrypted PHI audit log (replace stdout)	P0	2d
Dashboard auth (JWT + clinic roles)	P0	3d
Production deploy + HTTPS + domain	P0	2d
Stripe subscription (1 tier)	P1	3d
SMS appointment confirmation (Twilio)	P1	1d
After-hours flow (business_hours config)	P1	2d
Call transfer to on-call staff	P1	1d
CI/CD pipeline (GitHub Actions)	P2	2d

Exit criteria:

1. Signed BAA with clinic; BAAs with all providers
2. First invoice paid via Stripe
3. 99% uptime over 2-week observation
4. PHI never logged to stdout; audit trail queryable

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5 Level 3 — Multi-Tenant SaaS (Months 2–4)

Goal: 10 clinics onboarded with minimal manual work.

Task	Est.
Tenant CRUD API + phone-number mapping	5d
Self-serve onboarding wizard (dashboard)	7d
Per-tenant billing + usage metering (minutes)	5d
Editable config UI (prompt, voice, hours)	5d
Call recordings + playback in dashboard	3d
Reschedule + cancel calendar tools	4d
PostgreSQL migration (from SQLite)	3d
Monitoring (Sentry + uptime)	2d
Multi-language (Spanish) pilot	5d

Exit criteria: New clinic live in <2 hours without developer; 10 paying tenants.

6 Level 4 — Scale (Months 4–8)

Goal: Differentiated product; reduce churn; expand ARPU.

- Analytics dashboard (call volume, booking conversion, after-hours capture)
- Outbound campaigns (recall, no-show recovery) via Vapi outbound API
- CRM connectors (HubSpot, Salesforce native)
- athenahealth / eClinicalWorks calendar integration
- White-label option (clinic branding on widget + voice)
- SLA tier (99.9% uptime guarantee)
- Evaluate LiveKit migration for high-volume cost savings

Exit criteria: 50+ clinics; \$25K+ MRR; <\$0.20/min blended cost at scale.

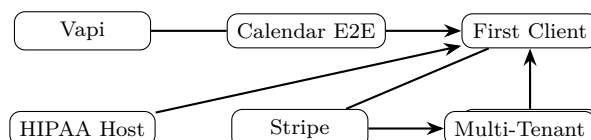
7 Level 5 — Enterprise (Months 8–18)

Goal: Multi-location groups; EHR-native; compliance audit-ready.

- Epic FHIR integration (appointments, patient lookup)
- SOC 2 Type II + HIPAA audit
- Dedicated CSM + custom contracts
- Multi-EHR per tenant (group practices)
- Insurance verification API integration
- Custom voice cloning per clinic (ElevenLabs Pro)

Exit criteria: 1 enterprise contract (\$10K+/mo); audit passed.

8 Dependency Graph



9 Milestone Timeline

Milestone	Target	Revenue
L1 Phone MVP	Week 2	\$0 (pilot)
L2 First paying client	Week 6	\$1.5–2.5K/mo
L3 Ten clinics	Month 4	\$15–25K MRR
L4 Scale	Month 8	\$50K+ MRR
L5 Enterprise	Month 18	\$100K+ MRR